

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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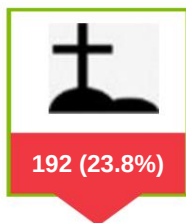
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°31/MVB_14/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (31)	- Ituri (20/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia. - North Kivu (10/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi and Mabalako. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 14, 2026
Publication date	June 15, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



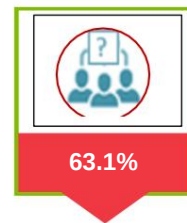
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as cases of tax evasion in the overall count.

Suspicious deaths occurring in the community are subject to investigation. further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Twenty -six (26) new confirmed cases, including 11 deaths, were reported during the day on June 14, 2026 in 2 provinces (Ituri: 21 and North Kivu: 5). These confirmed cases are distributed in the Mongbwalu (11), Nyankunde (4), Bunia (3), Rwampara (3) health zones in the province of Ituri and the Butembo (4) and Katwa (1) health zones for the province of North Kivu.

Eight (8) patients with Ebola virus disease have been declared recovered, including 3 in Bunia and 5 in Rwampara (province of Ituri).

- A meeting was held, chaired by the Director General of the INSP, with the response team, in the presence of the new military governor.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

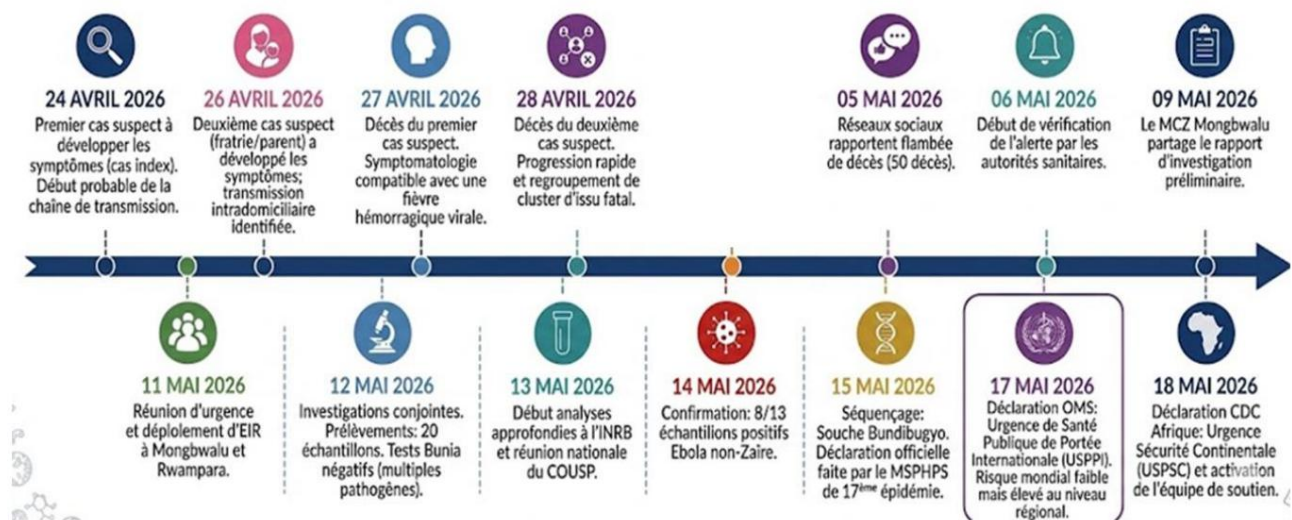


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 14, 2026

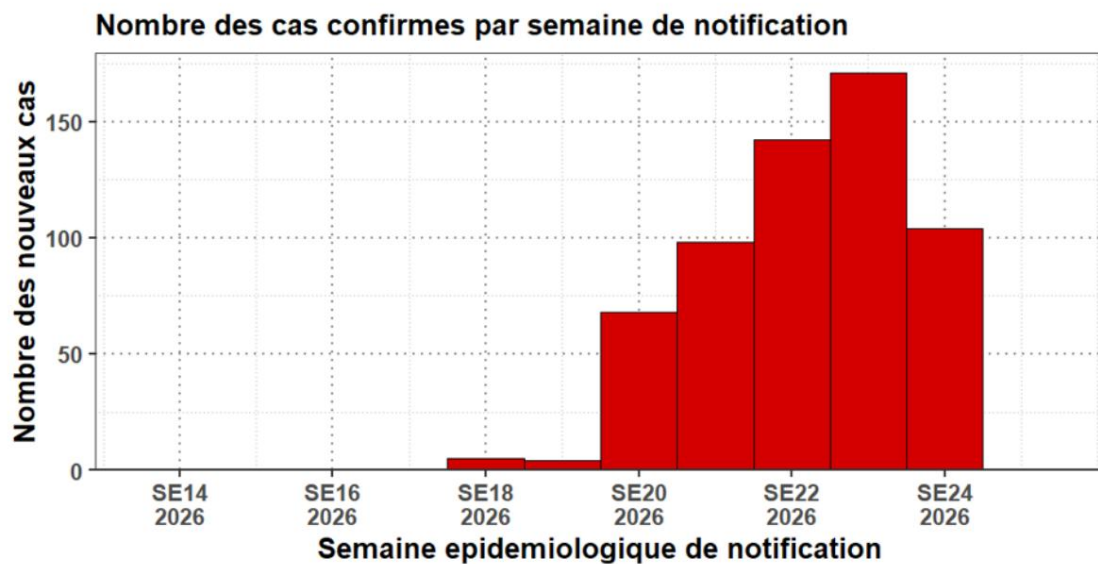
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	738	153	20.7%	20 out of 36 (55.5%)	21
North Kivu	67	38	56.7%	10 out of 34 (29.4%)	5
South Kivu			33.3%	1 out of 34 (2.9%)	0
Total	3 808	1 192	23.8%	31 out of 104 (29.8%)	26

- As of June 14, 2026, no new health zones were affected. The number of affected health zones remains at 31/104 (section 3.1). Approximately 91.3% of the confirmed cases reported in the three affected provinces are located in Ituri Province, making it the epicenter. Furthermore, North Kivu Province remains the most affected in terms of case fatality rate (56.7%), while Ituri Province shows a low case fatality rate (20.7%) despite the significant number of deaths recorded. South Kivu Province has not reported any confirmed cases since May 26, 2026.

3.2 Distribution by health zone (Cumulative as of 14/06/2026)

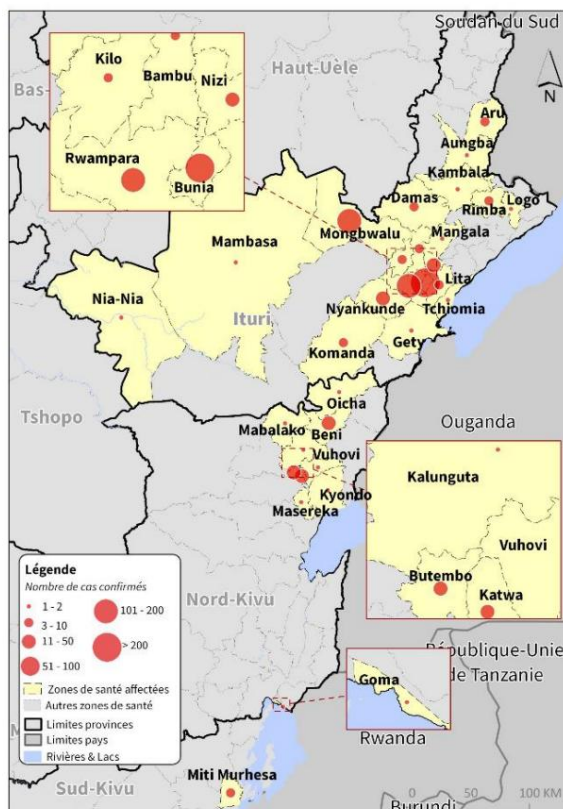
- Province of Ituri (738 cases):** Bunia (215), Rwampara (152), Mongbwalu (175), Nyankunde (42), Nizi (11), Bambu (7), Komanda (6), Lita (6), Mangala (5), Kilo (4), Damas (4), Rimba (3), Aru (3), Aungba (2), Mambasa (2), Logo (2), Tchomia (2), Gety (1), Kambala (1) and Nia-Nia (1 case). (Ninety-four (94) Additional cases are being ventilated.
- North Kivu Province (67 cases):** Katwa (25), Butembo (19), Beni (14), Oicha (2), Kyondo (2), Kalunguta (1), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- South Kivu Province (3 cases) :** Miti-Murhesa (3).

3.3 Time analysis: We observe an increasing number of confirmed cases from one week to the next, reflecting continued transmission of the disease in the community. A rapid geographical expansion of the epidemic is feared if public health measures are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 592.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 14, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 14, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia	215	20	9.3%
Rwampara	152	27	17.8%
Mongbwalu	175	80	45.7%
Nyankunde	42	4	9.5%
Bamboo	7	2	28.6%
Aru	3		33.3%
Kilo	4		25.0%
Nizi	11		9.1%
Mangala	5	1	60.0%
Damascus	4	1	0.0%
Aungba	2		50.0%
Gety		1	0.0%
Komanda	1	3	0.0%
Lita	6	0	0.0%
Logo	6	1	0.0%
Mambasa	2		50.0%
Rimba	2	0	0.0%
Tchomia	3	0	0.0%
Kambala			100.0%
Nia-Nia			100.0%
Other ZS (undisclosed data)	2 11 94	0 0 1 0 0 1 1 1 0	10.6%
Ituri subtotal	738	153	20.7%
NORTH KIVU			
Katwa	25	16	64.0%
Blessed	14	10	71.4%
Butembo	19	7	36.8%
Oicha	2	2	100.0%
Kalunguta			100.0%
Kyondo	1		50.0%
Goma		1	0.0%
Masereka	2 1 1	1	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
North Kivu subtotal	11 67	0 1 0 38	56.7%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	808	192	23.8%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- A meeting was held, chaired by the Director General of INSP/COUSP, with the response team, in the presence of the new Military Governor
- Closure of the training of chief medical officers of health zones and ACs for the Aru pool.
- Conclusion of the cross-border meeting between the DRC and Uganda on the coordination of response actions to the MVE.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 14, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		241	329	18	588
Alerts investigated		158	329	18	505
Investigation rate		65.6%	100.0%	100.0%	85.9%
Alerts confirmed	Alive	67	41	3	111
	Deceased	43	11	0	54
Total suspected cases for today		110	52	3	165

During the course of June 14, 2026, **588 alerts** were raised in the three affected provinces, of which **505 (85.9%)** were investigated. As a result, **165 suspected cases (54 deaths)** were recorded.

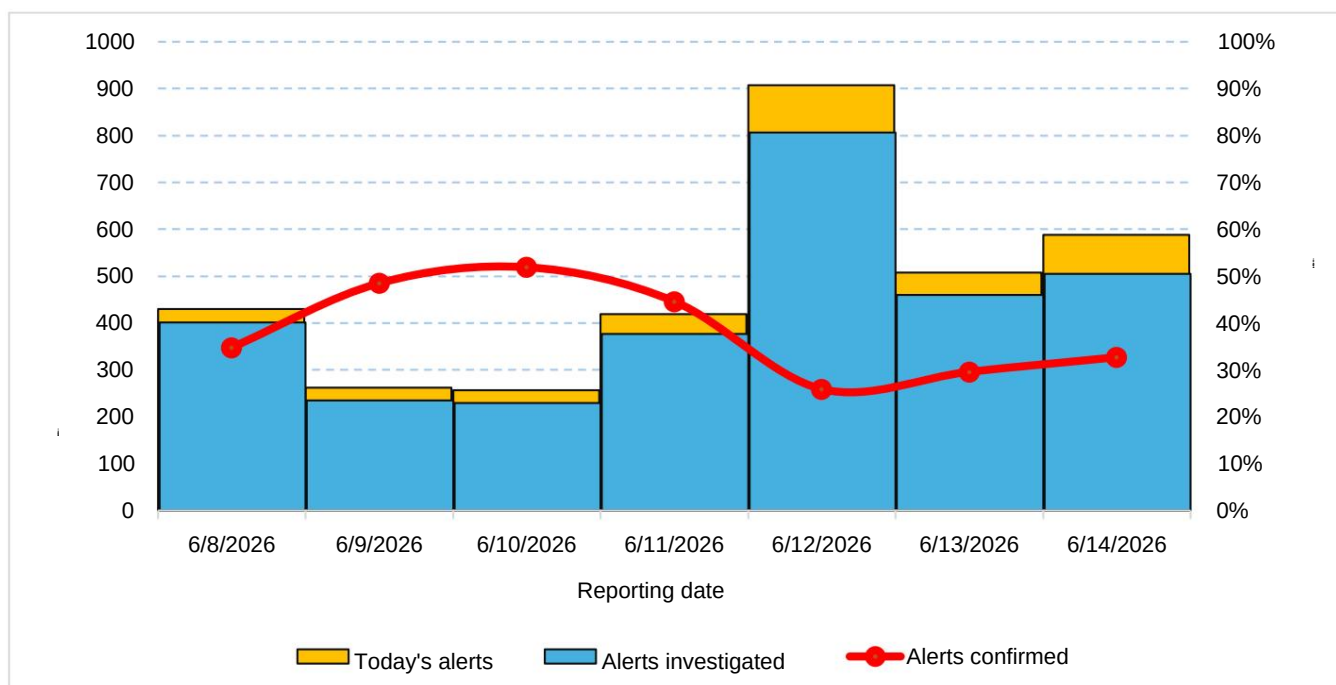


Figure 4. Evolution of alerts notified, investigated and the proportion of alerts validated during the last 7 days (June 8-14, 2026).

Table 3. Contact tracing of confirmed cases as of June 14, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	4,828	3,098	64.2%
North Kivu	1,212	684	56.4%
South Kivu	80	80	100.0%
Total	6 120	3 862	63.1%

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 314 new contacts were listed in 4 health zones of Ituri province.

Update on checkpoint and point of entry (PoC/PoE) activities

- Guided tour of the OFOO/DRC and Vurra/Uganda PoEs after the cross-border meeting.
- Supervision of activities at PoE Aéro Bunia, PoCs Mudzibala and Dele.
- Supervision of the construction work of the cabins.

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 14, 2026

Indicators	Results
Proportion of activated PoCs	60.0% (27/45)
Number of travelers who passed	47,369
through, % of travelers	94.0%
screened, % of travelers who washed their	92.6%
hands, % of travelers made aware of the risks	94.0%
Number of alerts notified	7
Percentage of alerts investigated or bodies swabs within 24 hours	0.0%
Number of validated alerts	7
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 101 new samples collected and analyzed (positivity 20.8%; n=21).
- **South Kivu :** 3 samples collected and analyzed, all came back negative.
- **North Kivu :** 38 samples collected and analyzed (Bundibugyo strain); 5 came back positive (13.1%). A total of 66 samples remain pending.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Provision of a PCI kit for decontamination in the Mongbwalu health zone (BCZ and ESS); briefing of the 44 service providers in the various health districts of the Bunia health zone on general information about Ebola virus disease and washing

hand hygiene; briefing of 14 service providers in the Mambasa health zone on standard precautions; decontamination of households of confirmed cases in the Rwampara, Bunia and Bambu health zones.

- **North Kivu:** Meeting with partner Medair on the need to improve the triage circuit at Katwa General Referral Hospital, compliance with decontamination SOPs, donning and removal of PPE; briefing of 61 healthcare providers in the health zones of Butembo, Katwa, Beni, Goma and Karisimbi on various IPC themes in the context of Ebola; continued provision of IPC supplies to priority health facilities in the health zones of Butembo, Katwa and Beni; decontamination of 2/3 health facilities (Matanda Regional Health Center & Emmanuel d'Alzon Hospital) in the Butembo health zone; decontamination of Antabuse Hospital (gap of 3/6 health facilities remaining to be decontaminated) in the Katwa health zone; risk assessment of exposure of 5/10 providers at St. Joseph Hospital and Antabuse Hospital in the Katwa health zone: the 5 providers classified as high risk; supervision of CH Ia VERITE/ZS of Butembo which received 5 suspected cases; decontamination of two ESS: CHE Beni in the AS of Malepe and Thwamahavuka in the AS Kanzulinzuli/ZS of Beni; Evaluation of 6 ESSs using the scorecard tool in the Beni health zone.
- **South Kivu:** Development of the distribution plan for 4600Kg/8,652m3 of inputs (PPE and PCI) to the 15 priority health zones of South Kivu; evaluation of a scorecard; theoretical and practical briefing of 65 service providers on standard precautionary measures and the decontamination technique with 3 buckets; provision of 60 handwashing kits in 3 schools (EP Kabindi, EP Murhanga, EP CIBATI-COMBO).

4.5 Holistic care

- Eight (8) patients (Rwampara: 5, Bunia: 3) with Ebola virus disease have been declared cured after obtaining negative control tests.
- Nutritional monitoring and care of patients in all CTEs: 536 hot meals were offered (119 suspected, 103 confirmed, 281 PPL and 33 accompanying persons).

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 14, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	312	42	7	361
Total admissions (24h)	44	8	2	54
Exits (24h)				
Deceased (Suspects/Confessions)	9	0	0	9
Non-	25	6	3	34
cases,	8	0	0	8
Cured, Escaped (Suspects/Conf.)		0	0	1
Transferred to the HGR	10	0	0	0
Total outflows	43	6	3	52
Patients in isolation (End of Day)	313	44	6	363
including confirmed (NC+AC)	140	14	1	155
and suspected cases	173	30	5	208
Overall occupancy rate	94.8%	62.0%	18.8%	83.8%

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 14, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	7 (100%)	0	3 (20%)
Confirmed cases currently being monitored that have benefited from SMSPS interventions	35 (42.7%)	1 (100%)	5 (25%)
New suspected cases that have benefited from interventions SMSPS	13 (59.1%)	0	7 (35%)
Suspected cases under follow-up that have benefited from SMSPS interventions	46(58,2)	6 (100%)	10(47.6)
Laboratory results have been announced	17	0	8
Including positive ones	8	0	4
Number of children separated in daycare who benefited from SMSPS interventions	1 (100%)	1 (100%)	3
Number of children separated in the community who benefited from SMSPS interventions	0	4	8
Number of accompanying persons at the CTE who benefited from SMSPS interventions	62	8	30
Number of PPLs who benefited from SMSPS interventions	6	5	0
Number of household and community members who benefited from SMSPS interventions	38	7	7
Number of EDS supported	0	0	0

4.6 Continuity of services.

- 11 recovery plans for HGR and CSR
- Preparation of drug needs assessments for ESS.

4.7 Risk communication and community engagement

North Kivu: 210 (11 men and 199 women) Adventist faithful from 7 districts sensitized in the Matanda parish on preventive measures against Ebola Virus Disease (EVD), risk communication and reporting of community alerts, referral of sick people to health facilities; 35 young people from the INTERADES association and students briefed on EVD and the management of rumors in the Matanda health district; 2 reservations overcome related to the transfer of patients in the Butembo health zone; 159 people (67 men and 92 women) reached by the awareness-raising on preventive measures against EVD during home visits in the Muyisa health district; 415 members (270 women and 145 men) of the Polyglossa/CBCA church in Butembo sensitized on preventive measures against EVD under the theme. "The responsible behavior of the Christian of the Plyoussa church for the fight against Ebola: no useless resistance, Ebola exists.

South Kivu: 3 radio stations broadcast Ebola prevention messages, 3 religious leaders engaged, 477 households visited, 1,480 people sensitized within households; mass awareness campaign with 304 members of the CEPAC Buloho church on prevention measures and signs of Ebola; advocacy with the pastor

from the CEPAC church so that he regularly raises awareness among the faithful of his church about Ebola prevention measures.

Ituri: 43 family members of 9 new admissions to the Nyankunde Ebola Treatment Center/Medical Ebola Center; interviews with members of seven families. **Results :** nine samples collected without refusal and 5 resistances to the EDS (Ebola Detection System) overcome; orientation meeting of 14 opinion leaders and group leaders tasked with supporting the prevention and management of community incidents alongside their deployment; mass awareness campaigns, including 175 men and 311 women, on Ebola prevention measures in various settings across the different affected health zones in Ituri; 4,615 people (1,823 men and 2,792 women) reached during 60 home visits; 4 mass awareness campaigns on Ebola prevention measures organized in 6 health zones, reaching 963 people; 43 people briefed (31 traditional healers and 12 influencers) in the Bunia, Mongbwalu, and Rwampara health zones.

Infodemic Management/Feedback: It has become a recurring issue for EDS teams to arrive in very small numbers at burial sites, and community members are forced to assist them without protection, risking infection. Action: Responses and clarifications provided to communities.

Support to other pillars : Surveillance : Support for listing the 14 contacts around 5 suspected cases (AS Mongbwalu, ZS Mongbwalu); **PCI :** preparing the family for joining the EDS (AS Dele/ZS Rwampara).

Result : EDS completed; Support provided: guidance for 6 patients admitted to the CTE CME Nyankunde and awareness-raising for 26 people accompanying them.

4.8 Logistics and Security

- Receipt of 9 Radione machines and reagents.
- Delivery of PCI inputs.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.
- Validation of the evaluation form for experts leaving the Ebola outbreak zone.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued awareness-raising efforts among local communities regarding complaint mechanisms.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3 Weakness in contact tracing (overall rate 63.1%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the provinces of Ituri and South Kivu	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Responsible pillar	Due date
1	Strengthen awareness-raising and community engagement activities of local leaders	CREC	Continue
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continue
5	Catch-up in contact tracing in Ituri remains weak.	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory		Day 2

Guided tour of social media influencers from Bunia and Mongbwalu at the Rwampara CTE.



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